INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00415
Version 5

Reference calendar for daily medicine and reviews

Treatment stages

01 **Adjuvant oral treatment**
12-month planned course

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	12
04	Sources and further information	19

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma · National Cancer Control Programme (NCCP) 00415

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

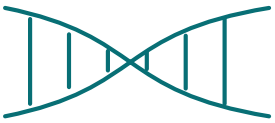
Medicines in this regimen

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

ORAL BRAF INHIBITOR

Dabrafenib

Tafinlar



Blocks altered BRAF signalling

Dabrafenib inhibits an altered BRAF protein that sends growth signals inside eligible melanoma cells. Trametinib blocks another step in that pathway. The tumour's BRAF result is checked before selecting this treatment.

BRAF V600 result → BRAF blocked → Growth signal reduced

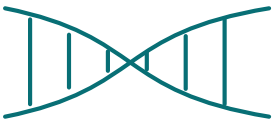
What to watch for

Fever needs the written hold/contact plan. Check other medicines and contraception because interactions can matter.

ORAL MEK INHIBITOR

Trametinib

Mekinist



Blocks downstream MEK signalling

Trametinib inhibits MEK proteins, which pass growth signals onward inside cells. It is used with dabrafenib in this plan. Normal tissues can also be affected, so heart checks and prompt reporting of eye or breathing symptoms remain important.

MEK signal blocked → Works with dabrafenib → Heart / eye monitoring

What to watch for

Attend cardiac tests. New eye symptoms, cough, breathlessness, swelling or severe diarrhoea need prompt review.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Record the original melanoma site and pathology findings. Breslow thickness and ulceration describe features of the skin tumour; stage describes its extent. Melanoma arising in the eye or a mucosal surface may need a different pathway.

BRAF and other tumour tests

A BRAF V600 result identifies whether the targeted medicines in this plan can act on the tumour. Keep the laboratory result and date. A tumour change is not, on its own, evidence of an inherited condition.

Surgery, radiotherapy and medicine have different roles

Ask whether medicine is intended to reduce recurrence after complete surgery or control cancer that remains. Add any operation or radiotherapy to the plan with its own purpose, timing and risks.

Stage and response are different

Stage describes the extent of disease. Response describes a change in measurable cancer during treatment. After complete removal there may be no visible tumour to measure; follow-up then looks for recurrence and treatment effects.

A new skin change still needs review

Report a new or changing mole, a sore that persists, a new lump or an unexplained symptom. A photo or a good response in one area cannot rule out a separate problem.

Your medicines and the overall plan

The schedule in this setting

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

How the treatment works

Dabrafenib inhibits an altered BRAF protein that sends growth signals inside eligible melanoma cells. Trametinib blocks another step in that pathway. The tumour's BRAF result is checked before selecting this treatment. Trametinib inhibits MEK proteins, which pass growth signals onward inside cells. It is used with dabrafenib in this plan. Normal tissues can also be affected, so heart checks and prompt reporting of eye or breathing symptoms remain important.

The role of earlier or later treatment

Ask how the medicine fits with previous or planned local treatment. The timing of an operation, radiotherapy and a tablet course should be written in one agreed plan.

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what outcome this treatment aims to improve for your diagnosis and what alternatives exist. Results from a study group help a discussion but cannot promise your own result.

Alternatives and review points

Agree the next decision point and what would make the plan change. Ask about other suitable treatments, trials and symptom care. Support for comfort and function can continue alongside cancer treatment.

Help with daily life

Tell the team what is hardest in daily life: travel, work, caring duties, costs, sleep or confidence. Ask who can help and how to contact them.

Shared understanding

Before going home, check that you can describe your medicine schedule, next tests and urgent-contact route. Ask for an interpreter, larger print or a supporter if that would help.

What the evidence means for your treatment

Discuss the evidence for your diagnosis, tumour test results and treatment setting.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

SACTCHECK · CHAPTER 02

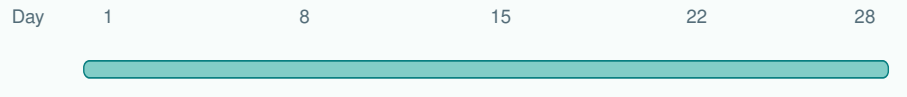
Your schedule, tests and appointments

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma · NCCP 00415

The overall treatment plan

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

12-MONTH PLANNED COURSE

Adjuvant oral treatment

Planned 12 months; regular reviews

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00415 v5: current regimen source

THE WHOLE TREATMENT JOURNEY

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma: treatment and checks

NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00415	Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/liver tests, electrocardiogram (ECG), echocardiogram, skin examination and eye assessment when indicated.	Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest computed tomography (CT) every six months are specified surveillance checks; disease-response imaging follows the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Cancer, skin and daily function NCCP 00415	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00415	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

What each check is for

1 Adjuvant oral treatment

12-month planned course

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

Ask / **act:** Confirm the current treatment phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Swallow the prescribed capsules/tablet whole with water. Take each dose at least one hour before food or at least two hours after food. Take the once-daily trametinib dose at the same time each day, alongside either the morning or evening dabrafenib dose according to your written plan. Keep the prescribed storage instructions.

STEP 2

Missed, vomited or held treatment

The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

STEP 3

Appointments and supply

Plan blood tests, travel and medicine collection in advance. If you need an interpreter or someone to help at home, tell the team before the appointment.

STEP 4

Paper copies and contact information

Ask for your latest schedule and contact details on paper. Replace old versions when treatment changes so everyone helping you uses the same plan.

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Practical preparation

Bring your medicine list, allergy information and questions. Tell the team about practical problems with travel, swallowing, wound care or remembering doses before treatment starts.

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.

Continuing medicine precautions

Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

The complete paper version

Use this handbook as your paper record. Keep the current schedule, medicines, tests and contacts together, and replace outdated pages after a change. The matching QR link is optional.

Symptoms and supportive care

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma · NCCP 00415

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily oral treatment	 **Days 1–28** Both medicines daily; adjuvant course planned for 12 months with regular safety reviews.
Safety reviews as scheduled	 **Days 1–28** Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.
Symptoms and support	 **Days 1–28** Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

Timing note 1: Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Timing note 2: The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

Timing note 3: Advanced and adjuvant treatment are distinct; do not apply a 12-month endpoint to all patients.

Timing note 4: Fever holds both drugs; other toxicities may have component-specific rules. The treating team decides restart and dose changes.

Timing note 5: The historical PDF contains adjuvant00415b while the reviewed catalogue excerpt lists00415a; current source reconciliation must resolve/record this difference.

Timing note 6: Regular skin reviews are required; ask the team to confirm the interval and keep the recorded appointments.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00415 v5: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–28: Every day in the reference calendar

Both medicines daily; adjuvant course planned for 12 months with regular safety reviews. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

Support around the medicines

1 Fever action plan

If temperature reaches 38°C or higher, withhold both dabrafenib and trametinib and contact your oncology team promptly for assessment. For recurrent episodes, the team may advise withholding at the first symptom. Restarting needs the team's plan after at least 24 hours without symptoms; do not use this as an automatic self-restart rule.

2 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

3 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

4 Pregnancy and fertility precautions

Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

5 Eating, activity and practical help

Let the team know if cramps, altered taste or poor appetite make eating or moving difficult. Ask for practical food and activity advice that fits your symptoms and any other illness.

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/liver tests, ECG, echocardiogram, skin examination and eye assessment when indicated.

During treatment

Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan.

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Getting the result

Keep a named contact for bookings and results. If an expected appointment or result is missing, call that service; do not take silence as confirmation that everything is complete.

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Eating, drinking and activity

Report nausea, mouth discomfort, diarrhoea or weight loss before they make eating and drinking difficult. The team can adjust symptom support and arrange dietitian or rehabilitation advice.

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Protect exposed skin with clothing, a hat and shade. Use broad-spectrum sunscreen with sun protection factor (SPF) 30 or higher and reapply as directed. Avoid sunbeds. Cloudy weather does not remove ultraviolet exposure.

Know the pattern of your skin

Ask the team to show you how to check your skin and treated area. Use a mirror or a helper for areas you cannot see. Report a new lump, a changing lesion or a sore that does not heal.

Photographs are optional

A private, dated photograph can help describe a change if your team finds it useful. Keep the lighting and size reference consistent. Photographs support a discussion; they do not replace examination and need not be uploaded.

Wounds, nails and sensitive skin

Follow the wound and skin-care instructions for the treated area. Ask before adding a cream or remedy. Increasing redness, discharge, pain, bleeding or difficulty using the area needs assessment.

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.



Medicine-specific precautions

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner’s pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Questions worth asking

When are my next tests?

Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required: ask the team to document the interval because the source gives monthly and 60-day instructions in different sections. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

Your diagnosis and treatment setting NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Adjuvant oral treatment

Active medicines: Dabrafenib + Trametinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Fever ≥38°C: hold both medicines and contact the team; restarting requires the agreed plan after at least 24 hours symptom-free. Recurrent episodes need their own assessment. Dabrafenib is moderate-to-high emetic risk and trametinib minimal-to-low; the combined plan must reflect both.

- Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.
- Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fever / chills

Both medicines

At 38°C or above, hold both medicines and contact the team under your fever plan. Infection and dehydration must also be considered.

2 Eyes / vision

Dabrafenib / trametinib

Report blurred vision, eye pain, redness, light sensitivity or a change in sight promptly; some eye problems need urgent specialist treatment.

3 Heart / circulation

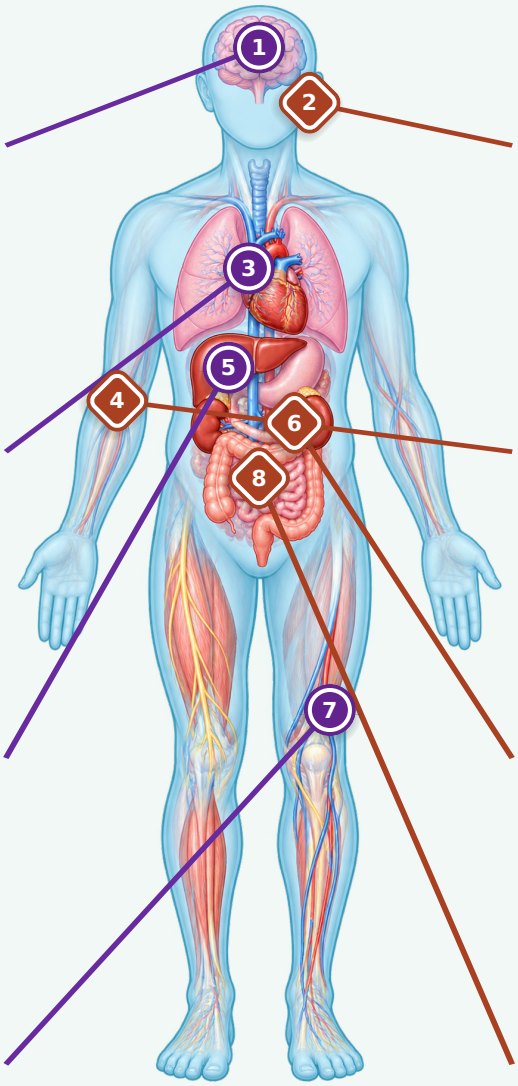
Trametinib / combination

Keep ECG and heart ultrasound checks. New breathlessness, swollen ankles, palpitations or chest pain needs prompt assessment.

4 Skin / new lesions

Both medicines

Rash and skin sensitivity may occur; new changing spots need review. Blistering, peeling or mouth sores with a rash needs urgent help.



5 Liver

Both medicines

Liver changes may first appear in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Kidneys / fluids

Both medicines / fever

Fever, vomiting or diarrhoea can cause dehydration and kidney problems. Call for reduced urine, dizziness or inability to keep fluids down.

7 Muscles / joints

Both medicines

Aches can occur. Severe muscle pain or weakness, especially with dark urine, needs urgent advice and possible blood tests.

8 Stomach / bowels

Both medicines

Tell the team about persistent diarrhoea, vomiting or pain. Severe abdominal pain, blood or black stools needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

Sources and further information

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma · National Cancer Control Programme (NCCP) 00415

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00415 v5: current regimen source**
   https://healthservice.hse.ie/documents/6732/415_Trametinib_and_DabrafenibTherapy.pdf
2. **EMA: Mekinist medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/mekinist
3. **EMA: Tafinlar medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/tafinlar
4. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
5. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
6. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
8. **NCI: skin cancer — patient information**
   https://www.cancer.gov/types/skin/patient/skin-treatment-pdq
9. **HSE SunSmart: protection from sun and sunbeds**
   https://www.hse.ie/conditions/cancer/protect-skin-sun-sunbeds/